

ED VTE RISK ASSESSMENT

Addressograph

STEP ONE**Mobility – all patients** (tick one box)

Medical patient **NOT** expected to have significantly reduced mobility relative to normal state*
 (*Significantly reduced mobility: bed bound, unable to walk unaided or likely to spend a substantial proportion of their day in bed or in a chair)



Medical patient expected to have on-going reduced mobility relative to normal state (*see left)



Surgical Patient

**Assessment complete****Assess for thrombosis and bleeding risk below****STEP TWO****Thrombosis Risk - Any tick for thrombosis risk indicates INCREASED risk of VTE****Patient Related****Admission Related**

Active cancer or cancer treatment

Significantly reduced mobility (*see above left)

Age >60

Hip or knee replacement

Dehydration

Hip fracture

Known thrombophilia (aka: hypercoagulability / prothrombotic state – an abnormal tendency to develop blood clots)

Anticipated total anaesthetic + surgery time likely ≥90 minutes

One or more significant medical co morbidities (e.g. heart disease, metabolic, endocrine or respiratory pathologies, acute infections, inflammatory conditions)

Anticipated surgery involving pelvis or lower limb with total anaesthetic + surgery time likely ≥60 minutes

Obesity (BMI>30 kg/m²)**NB: Patients ≥150kg require 10,000 units Dalteparin od**

Acute surgical admission with inflammatory or intra-abdominal condition

Personal history/first degree relative with history of VTE

Critical care admission

Use of hormone replacement therapy

Anticipated surgery with significant reduction in mobility

Use of oestrogen-containing oral contraceptive pill

Other

Varicose veins with phlebitis

Pregnancy or less than 6 weeks post-partum; see UHL 'Thrombosis in pregnancy...' guideline and assessment form

No identifiable risk factors**Bleeding Risk - any tick should prompt clinical staff to consider if bleeding risk is sufficient to preclude pharmacological thromboprophylaxis****Patient Related****Patient Related**

Active bleeding

Acquired bleeding disorder e.g. acute liver failure

Significant traumatic spinal injury

Acute Stroke / Significant head injury

Concurrent use of anticoagulants known to increase risk of bleeding e.g. Warfarin with INR >2.0

Untreated inherited bleeding disorder e.g. von Willebrand's disease

Platelets <75x10⁹/l, check on admission

Uncontrolled hypertension (>230/120)

Admission related

Neurosurgery, spinal surgery or eye surgery

Other:

Other procedures associated with high bleeding risk

Lumbar puncture/epidural/spinal anaesthesia expected within the next 12 hours **OR** performed within the previous 4 hours**No identifiable risk factors****For operational reasons the need for mechanical thromboprophylaxis is not assessed in ED and must be considered during reassessment on admission.****The patient information leaflet should also be provided on arrival on the ward****STEP THREE**

Increased risk of VTE with low bleeding risk: prescribe DALTEPARIN 5000 units od (currently replaced with enoxaparin 40mg)



Increased risk of VTE with significant bleeding risk:
 Low molecular weight heparin contraindicated at this time



Low risk of VTE:
 Early mobilisation

**Increased risk of VTE**

If eGFR <30ml/min: 2500 units Dalteparin od (currently replaced with enoxaparin 20mg)

Not enough information available to safely complete form



Date and time:

Patient ≥150kg: 10,000 units Dalteparin od

Name:

Signature